

Governance for public health across the health and allied sectors

A report to guide country-level institutional capacity for essential public health functions underpinning multisectoral approaches



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Organization**

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Abbreviations

COVID-19	coronavirus disease
EPHF	essential public health function
IANPHI	International Association of National Public Health Institutes
IRIS	Institutional Repository for Information Sharing
NPHI	national public health institute
PRISMA	preferred reporting items for systematic reviews and meta-analyses
SDG	Sustainable Development Goal

This report is aimed at national and subnational health and non-health actors involved in public health governance and the coordination and delivery of essential public health functions and services.

1. Background



1.1 Challenges and needs for public health governance

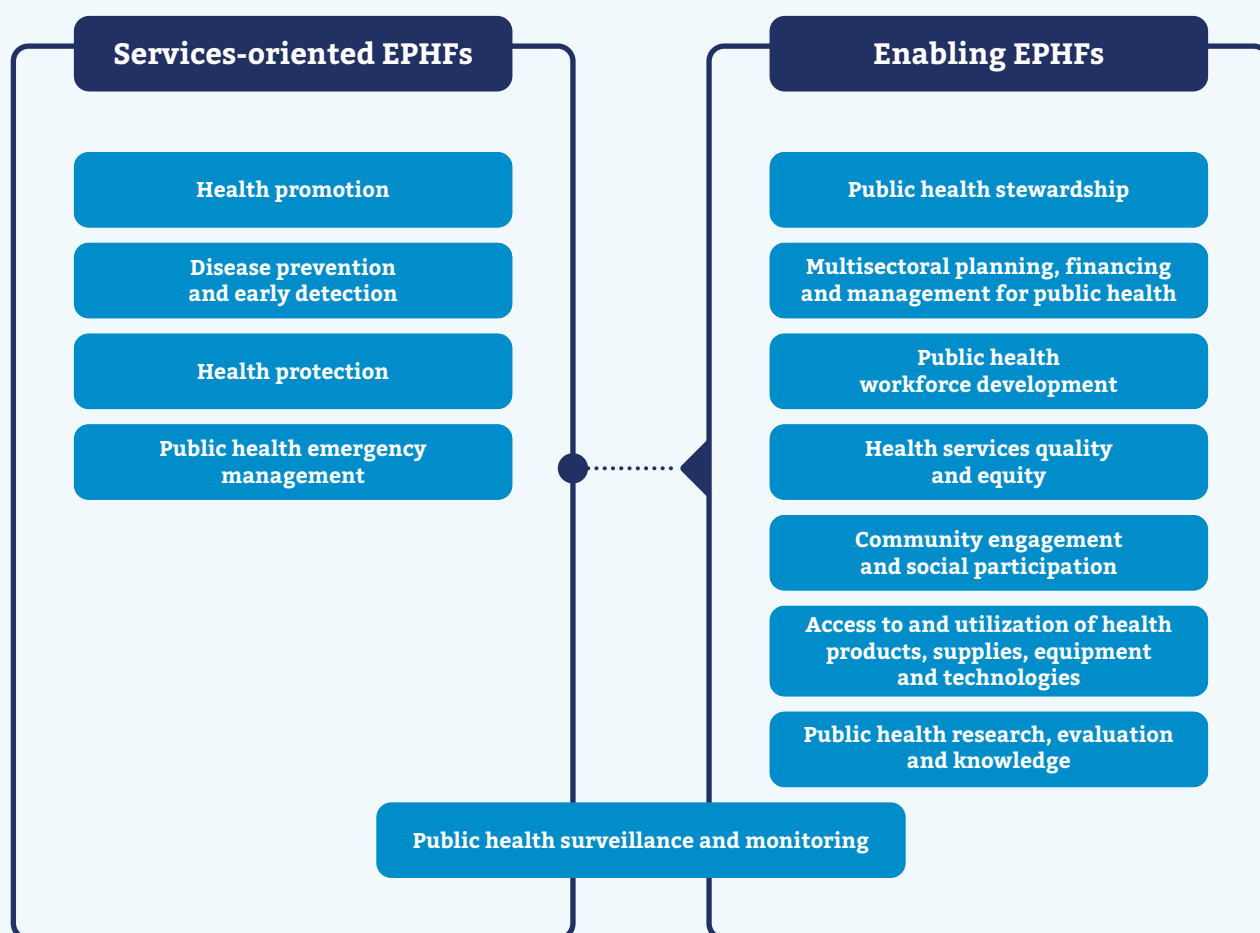
Health threats, ranging from pandemics such as coronavirus disease (COVID-19) to chronic stressors such as the rising prevalence of noncommunicable diseases, ageing populations, protracted armed conflict, fiscal constraints, and climate change, are challenging public health capacities and systems in all countries. The public health landscape is complex and country specific. It is influenced by factors both within and outside the health sector, such as changing demography, lifestyles, social equity and inequity, the environment, available social protection, intersectoral collaboration, and health system and institutional capacity to deliver the essential public health functions (EPHFs). For example, it is estimated that social determinants of health contribute to approximately 30–55% of health outcomes (1). Moreover, the workforce delivering the EPHFs is multisectoral and multidisciplinary involving the core public health personnel, health and care workers, and occupations allied to health (2, 3). The diverse landscape of public health necessitates multisectoral and integrated actions, supported by robust, well coordinated governance with participation of sectors and actors within and outside the core health sector.

There are indeed examples of intersectoral coordination for public health. In many instances, however, they are limited to large-scale emergency contexts, and are often reactive and time-bound. While Health in All Policies has been an aspiration for many years, its realization in planning, coordination, financing and service delivery remains inadequate. Moreover, current efforts to address health priorities are fragmented and siloed, hampering opportunities to build public health capacities and institutions (2). Health investments have been largely tailored to curative and individual clinical services and response capacities, and lack a proportionate focus on all areas of public health in line with the current and future burden of diseases and population health needs. This was evident in the disproportionate impact of COVID-19 on those socioeconomically marginalized and elderly people and with co-morbidities, including in countries with perceived high levels of health system maturity, universal health coverage and health security. Some of the major issues related to approaches to public health involve inadequate and fragmented public health governance and the lack of implementation of Health in all Policies (4), whole-of-government and whole-of-society approaches to public health.

The unified list of EPHFs developed by the World Health Organization (WHO) and the International Association of National Public Health Institutes (IANPHI) and endorsed by countries and partners provides a framework that represents the comprehensive scope of public health, encompassing both service-oriented and enabling public health functions (Figure 1) (5). The list spans multiple non-health sectors and helps to increase recognition of the economic, environmental and social determinants of health, conferring broad population benefits that might be underprovided without routine

operational intervention. Within the EPHFs, governance for public health is identified as an essential enabling function to deliver, coordinate and ensure accountability for public health through the entities designated or regulated by the State. Effective public health governance can therefore ensure more integrated and proportionate delivery of EPHFs and public health services as key to improving population health, building resilience and advancing universal health coverage, health security and all the Sustainable Development Goals (SDGs) (6).

Figure 1. Unified list of essential public health functions, encompassing service-oriented and enabling functions



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1.2 Aim, objectives and target audience

Employing the EPHFs as a framing for public health, the aim of this report is to provide guidance for country- and global-level public health actors to define and strengthen governance for public health for the delivery of EPHFs.

The objectives include to:

- collate the current understanding of public health governance and develop an operational definition;
- identify and describe the core components, enablers and guiding principles required to improve public health governance across health and other sectors responsible for various aspects of population health;

- promote the roles of the existing institutional set-up (including ministries of health, national public health institutes or their equivalent with heads of government) in applying the core components, guiding principles and enablers to governance for public health, considering the dispersed nature of public health stakeholders in countries.

This report is aimed at national and subnational health and non-health actors involved in public health governance and the coordination and delivery of essential public health functions and services, particularly including the governments, ministries of health, public health institutes and allied ministries with responsibility for and key roles in public health (for example, finance, environment, education, agriculture and commerce), as well as subnational authorities.

.....

1.3 Method

An initial scoping review was completed in 2024. This included a systematic search of relevant peer-reviewed literature using PubMed and Embase. The peer review search was supplemented by a grey literature search of the websites of the World Bank, (IANPHI, the WHO Institutional Repository for Information Sharing (IRIS) and Google Scholar. Search terms and inclusion and exclusion criteria are included in Annex 1. A technical report with findings was drafted.

In 2025, building on the scoping review and the technical report, the authors continued the analysis with a focus on defining and strengthening public health governance. An additional targeted search of examples and cases of public health governance operationalization was conducted. Results from the literature search are presented in a PRISMA (preferred reporting items for systematic reviews and meta-analyses) flow diagram in Annex 1.

2. Results



2.1 Defining public health governance

There were no explicit definitions of “public health governance” in the reviewed literature, but definitions of public health, governance, health governance, governance for health and well-being), or health systems governance appear more commonly (7). According to *The world health report 2000*, health systems can be understood “as comprising all the organizations, institutions and resources that are devoted to producing health actions” (8). Public health is defined as “an organized effort by society, primarily through its public institutions, to improve, promote, protect and restore the health of the population through collective action” (9). Governance in literature generally refers to rules, relationships and processes, formal or informal, that influence and coordinate the actions of decision-makers at different levels (9–11). Health system governance often refers to the processes, structures and institutions that are in place to oversee and manage a country’s health system (13–15). The lack of conceptualization and definition of public health governance is perhaps attributed to the complexity, overlaps and evolution of the understanding of health systems, public health,

and public health systems, in addition to the complexities inherent in understanding governance as it relates to institutions, systems and processes. Besides, intersectoral governance was also mentioned in literature, but no explicit definition was given (10). Some literature lists desired attributes of governance but does not provide a definition (11).

By definition, health system boundaries go beyond individual-focused health care to encompass population health and multisectoral actions for health (7, 16). Such an approach should ideally cover public health governance by also overseeing and managing public health functions and services (promotion, prevention, protection, emergency management at population level) rather than only focusing on individual health care (16). This means that health system governance that integrates public health governance would also encompass the role of multisectoral collaboration in routine contexts (17–22), not only during the management of public health crises, when the health sector is overwhelmed and other sectors and public

functions are at risk or impacted. While certain aspects of public health may be covered by health system governance in countries, the holistic inclusion of public health is often missing or unclear due to artificial divides between health systems focusing on health service delivery, or the dispersion of public health functions across health and allied sectors in many countries. This situation is also reflective of a country's set-up of various line ministries and departments in the delivery of respective public functions, in which the role of the ministry of health is defined in line with the delivery of health care services within its

jurisdiction. The definition of public health governance in this document takes into consideration the realities in countries while promoting more explicit, integrated, comprehensive and coordinated public health governance, taking into account the roles of stakeholders within health and allied sectors. In this regard, an operational definition of public health governance is presented in Box 1, based on literature and collective knowledge and experiences. Definitions of related terms are collated in Annex 2.

Box 1. Operational definition of governance for public health

Governance for public health refers to the processes, structures and mechanisms for decision-making and oversight to guide, coordinate and ensure accountability regarding the collective efforts of all stakeholders in the delivery of all essential public health functions and services within a designated context. It involves clear authority with agreed mandates, resource stewardship, operational and scientific independence, and collaborative arrangements across all levels within and beyond the health sector (19, 23–26).



»»» The definition of public health governance in this document takes into consideration the realities in countries while promoting more explicit, integrated, comprehensive and coordinated public health governance, taking into account the roles of stakeholders within health and allied sectors.

2.2 Stakeholder landscape in public health governance

Public health governance typically involves multiple actors from different sectors across society that plan, finance and implement EPHFs and public health services, including governmental and nongovernmental stakeholders in health and allied sectors (Figure 2). While there can be variation between countries, stakeholders include a range of organizations at all administrative levels, including ministries of health and national public health institutes; allied ministries (such as education, environment and climate change, food and agriculture, labour, finance, animal husbandry and veterinary services, housing and urban affairs, rural and community development, water sanitation and hygiene, disaster management, and transport and road safety); subnational stakeholders, including local administrative authorities and communities; civil society organizations; international organizations; national associations of public health; schools of public health; and the private sector.

For example, tackling a public health challenge, such as flooding in a country, would involve a wide range of national and local authorities directly and indirectly responsible for public health, including the ministry of environment, civil defence, meteorological office, local authorities, urban planning authorities, fire and rescue services, public health institutes, water and transport ministries and community groups. Public health information and communications provides another example: entities in the health sector are responsible for generating knowledge and evidence-based advice, and tailoring communications to the requirements of decision-makers, allied sectors and the public, while entities in allied sectors play a key role in utilizing public health information in policies and planning for positive health impact, as well as supporting communication of public health information and knowledge to the public. Figure 2 shows examples of stakeholders that could have key roles in public health governance at national and subnational levels.

In Ireland, for example, the national-level structures involved in public health governance for delivering health promotion and disease prevention include the Department of Health as well as a number of departments in allied sectors, such as the Department of Agriculture, Food and the Marine, and the Department of Employment Affairs and Social Protection (Figure 3) ([27](#)).

Public health governance typically involves multiple actors from different sectors across society that plan, finance and implement EPHFs and public health services.

Figure 2. Examples of typical country-level stakeholders involved in public health governance across health and allied sectors

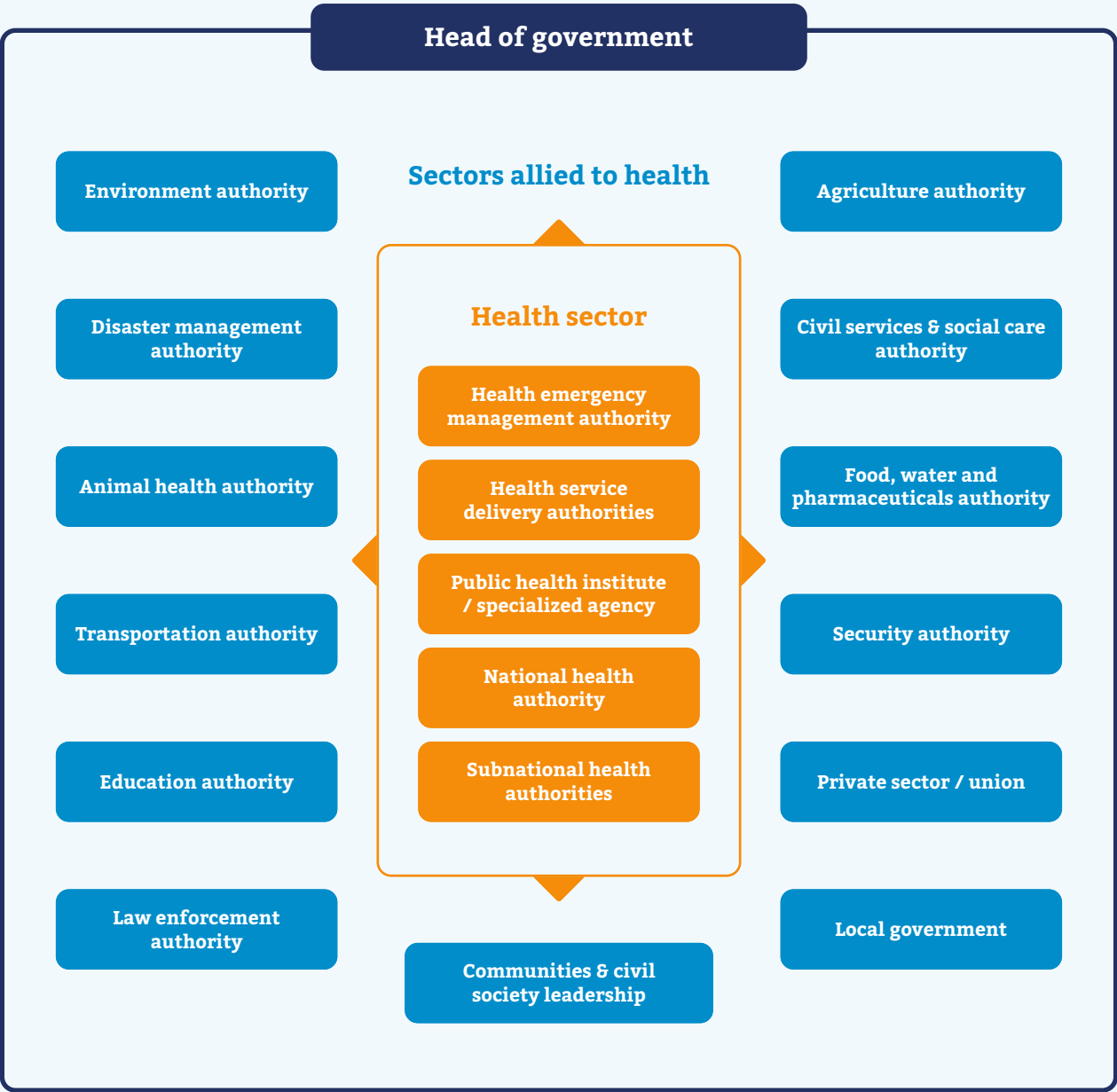
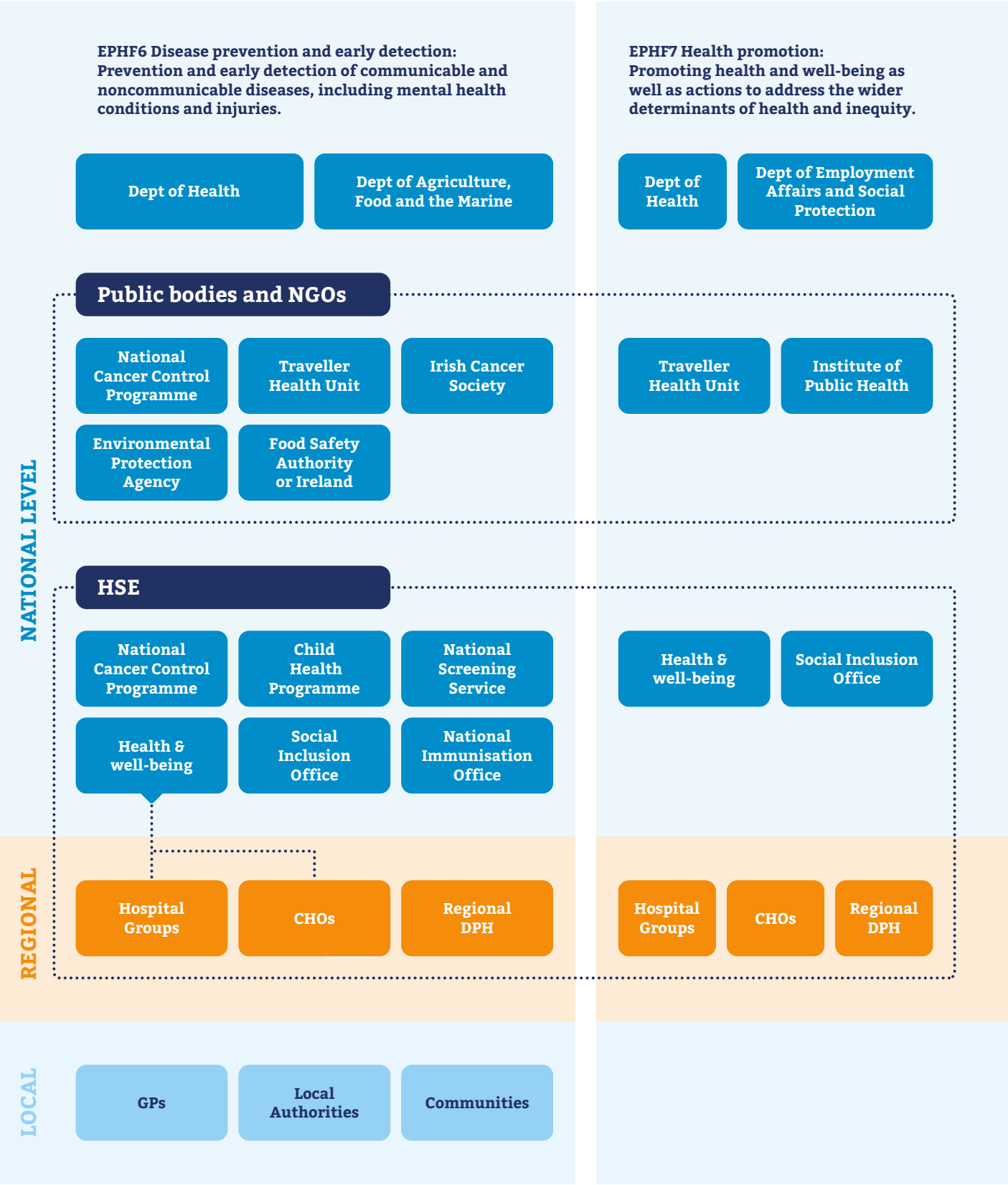


Figure 3. Illustrative structures supporting the delivery of health promotion and disease prevention public health functions in Ireland



EPHF: essential public health function; Dept: department; NGOs: non-governmental organization; HSE: Health Service Executive, Ireland; CHOs: community health organization; DPH: Department of Public Health; GPs: general practitioners.

Literature references show that ministries of health and national public health institutes (NPHIs) commonly play central roles in public health governance. Ministries of health often provide oversight and policy responsibility for the health sector, and stewardship of strategic public health action in partnership with other sectors and government departments. While NPHIs or similar organizations reporting to the ministry of health have historically held primary responsibility for public health, with a focus on health security and protection for emergency preparedness and response, that have not always performed adequately in matters relating to health promotion, public health stewardship, disease prevention or multisectoral coordination with allied sectors playing crucial roles in public health (28, 29).

By scope, responsibilities for public health governance are usually dispersed among various institutions. For example, in the United Kingdom, the United Kingdom Health Security Agency is responsible for health security; the Department of Health and Social Care (ministry of health equivalent) is responsible for health prevention

and promotion; and water and sanitation lies under the Drinking Water Inspectorate, local administration and the Water Services Regulation Authority (Ofwat) (30). Governance is typically vertical within a given institution or specific to delivery of certain EPHFs with other entities. However, the governance roles of the various responsible sectors are usually unclear and uncoordinated in integrated delivery of EPHFs. This lack of coordination has been attributed to the absence of a designated and empowered coordinating entity and the lack of acknowledgement of the interdependence of all the EPHFs, particularly during periods of normalcy. However, even where a national public health authority has been identified to lead public health, it may not have the adequate mandate, capacity or legislative power to coordinate the full range of EPHFs, as in the case of many NPHIs. The ministry of health (in many cases, the parent body of NPHIs) may also be limited in mandate to ensure that other sectors are adequately accountable in the delivery of their respective roles in EPHFs.

2.3 Core components, enablers and guiding principles for public health governance

Despite varying approaches to public health governance and actors involved, several key components, enablers and guiding principles of effective public health governance have been identified (25, 26). Strong public health governance should be responsible for oversight, coordination and partner engagement, resource stewardship, monitoring and evaluation, and strategy and policy development, as well as having the legislative power for effective operationalization and delivery of EPHFs.

Additionally, governing bodies should be enabled to perform these functions through legislation explicitly stating their role and authority in public health governance, adequate financial support, strong institutional structures, and sustained political will (Table 1) (5, 25, 26, 31, 32).

Table 1. Enablers, core components and guiding principles of effective public health governance

Element	Description	Examples
Enablers		
Legislation	A legal mandate is in place authorizing a government entity to be responsible for management, coordination and delivery of essential public health functions, with an explicit description of its role in agreement with other public health stakeholders.	The mandate of the National Public Health Institute of Liberia is built upon the National Public Health Institute of Liberia Act (2016) (33). The Law on Basic Medical and Health Care and the Promotion of Health in China defines the roles of government at all levels and medical and health care institutions in basic public health services and basic medical services (34).
Financing	Public health is sustainably financed within the designated entity for public health and allied sectors in line with the current and future burden of diseases and population health risks.	In Germany, statutory health insurance is the primary financing mechanism for health promotion and disease prevention (35). Statutory health insurance funds are collected through income-dependent contributions and a government subsidy, and then pooled and reallocated through the National Health Fund. The 2015 Act to Strengthen Health Promotion and Prevention requires statutory health insurance funds to invest a minimum amount annually in disease prevention and health promotion activities.
Institutional structures	Roles and responsibilities are clearly defined, with appropriate mechanisms to ensure accountability for health and allied sector actors.	In Japan, the organizational charts and roles of the Cabinet, Ministry of Health, Labour and Welfare, and public health entities and centres are clearly described and accessible, underpinned by a series of public health legislations and regulations, such as the Health Promotion Act and the Community Health Act (36, 37).
Political will	An enabling political environment facilitates orientation to public health by all relevant sectors, underpinning a Health in All Policies approach.	The Government of South Australia developed a cross-government strategic plan, which articulates priority areas (including safe communities, healthy neighbourhoods; creating a vibrant city; an affordable place to live; every chance for every child; etc.) for action across all government portfolios and provides a blueprint for action on the determinants of health (38). Under the Public Health Partner Authorities mechanism, for example, the Department for Health and Ageing and the Department of Environment, Water and Natural Resources collaborated on increasing access to parks and supporting the health and well-being benefits of contact with nature.

Element	Description	Examples
Core components		
Strategy and policy development	Responsible entities lead and contribute to development of policies, strategies and priorities to promote, protect and advance public health within the health sector and in allied sectors. This could entail defined input in relevant planning and budgetary processes of the ministry of health and allied sectors identified for their role in public health.	In Ireland, the Department of Health has the function of serving the public and supporting the Minister of Health, Ministers of State, and the Government by providing leadership and policy direction for the health sector with the objective of improving health outcomes and meeting the future needs of the population. The department launches public consultations to engage the public and stakeholders on public health-related regulations and policies (39).
Oversight	The governing or lead body has responsibility for implementation of essential public health functions and services, providing leadership and guidance to implementing agencies and delivery platforms.	In the Dominican Republic, in the context of an exercise to evaluate and strengthen the EPHFs pursuant to a decree by the Ministry of Public Health and Social Security (with the aim of informing national health sector development initiatives), a team was formed, led by the Vice-Ministry of Collective Health and coordinated by the Directorate of Population Health Management, to supervise and coordinate the implementation of the EPHF approach (40).
Partnership	Trusting relationships are built, managed and maintained with stakeholders within health and allied sectors from global to national, subnational and community levels, for example through engagement in and co-design of public health policies, planning, implementation, and monitoring and evaluation.	In Thailand, missions and responsibilities of the Department of Health, Ministry of Public Health, include coordinating and cooperating with government agencies, the private sector, local government, partners, networks, and international partners in the field of health promotion (41).
Legal authority	The governing or lead body has legal authority to enact public health regulations and coordinate stakeholders based on need and scientific evidence.	In the Kingdom of the Netherlands, the Ministry of Health, Welfare and Sport is responsible for developing legislation and regulations on specific aspects of public health provided at the national level, and enacting certain public health legislations (42 , 43).
Resource stewardship	Availability and good management of resources is ensured to deliver essential public health functions in line with priority areas of public health challenges and service requirements.	In England, local authorities with the responsibility of undertaking public health functions obtain annual grants from the Department of Health and Social Care for spending on public health, with financial accountability in place (44 , 45).

Element	Description	Examples
Monitoring and evaluation	Accountability mechanisms are established, including through routinely setting measurable goals, and monitoring and evaluating progress in the delivery of EPHFs and improvements in population health status, with reporting back to heads of government.	The Public Health Agency of Sweden is responsible for public health reporting, including the national public health survey that collects cross-sectional data on health, lifestyle and living conditions through collaboration with the 21 Swedish regions. The purpose of the survey is to report on the state of health and preconditions of health of the population over time, in order to monitor national and regional public health policies (46).

Key principles

Integration	Governance arrangements are in place that promote and ensure synergies across public health functions and services, and are mainstreamed in the health system and allied sectors.	The Health Care Model for Well-Being, the new health care approach implemented by the Government of Mexico, promotes synergies between EPHFs and public health services within the health system. The second layer of the model shows the organization of health care services, including health promotion, disease prevention, medical care, rehabilitation and palliative care. The third layer presents the organization for collective health interventions, based on the EPHFs in the fourth layer. The National Public Health Service, acting as the operational arm of the Secretariat of Health, implements and executes the EPHFs (40).
Comprehensiveness	An approach is adopted that proportionately prioritizes the full spectrum of public health functions across all service delivery domains (from prevention, promotion, and protection to public health emergency management) and the enabling EPHFs (see Figure 1).	In El Salvador, the Ministry of Public Health led the development of its Action Plan for the Strengthening of the EPHFs, covering the whole scope of public health functions (40). The process involved contributions of more than 70 health sector institutions.
Shared responsibility, including multisectorality	Clear, collaborative, coordinated and institutionalized accountability of various stakeholders is assured within and beyond the health sector, including public and private entities, communities, and multiple sectors that contribute to public health, for example, the health, education, housing, transport, and environment sectors.	In El Salvador, the EPHF implementation strategy led by the Ministry of Public Health highlights promotion of joint work with key actors and strengthening coordination with the National Integrated Health System consisting of multisectoral actors (including Salvadoran Social Security Institute; Ministry of Education, Science and Technology; Ministry of National Defence; representatives of the private sector hospitals; etc.) and other sectoral and multisectoral policies (40).

2.4 Public health governance arrangements

The arrangements for public health governance vary between countries. More than half of countries globally have an NPHI or equivalent. As of 2025, IANPHI had over 120 member institutions in 107 countries (for example, Ethiopian Public Health Institute, National Public Health Institute of Liberia, Santé publique France, United Kingdom Health Security Agency, Norwegian Institute of Public Health, Robert Koch Institute, Germany, and National Institute of Public Health of Guinea-Bissau). These institutions have different levels of responsibility for some or all of the EPHFs, with the framework developed by IANPHI and WHO (33, 47) acting as an important reference document to define the public health functions. There are also countries with well acknowledged multiple NPHIs (for example, Canada and Japan) and devolved public health governance (for example, United Kingdom Health Security Agency, Public Health Wales, Public Health Scotland) who host multiple agencies leading delivery of a range of EPHFs (29). As such, the prevailing narrative around public health governance often focuses on the role of NPHIs, in line with the functions and authorities defined and delegated by their parent organization.

In some countries, the primary responsible entity for public health is the ministry of health or equivalent. In this case, in addition to holding primary responsibility for governance of health service delivery (often focusing on individual clinical care), the ministry of health would also be, in principle, responsible for the delivery of the public health functions and public health services (for example, Ministry of Public Health, Qatar; Ministry of Public Health, Thailand; and Secretaría de Salud, Mexico). However, regardless of institutional set-up, due to the dispersed nature of public health within and beyond the health sector, the scope of public health governance is mostly limited to the role of the core health sector (that is, the ministry of health or NPHIs reporting to the ministry of health).

Based on the institutional set-up of and relationship between the ministry of health and the NPHI, institutional structural arrangements for public health governance in countries can be summarized into four broad types (Table 2).

»»» The arrangements for public health governance vary between countries. The prevailing narrative around public health governance often focuses on the role of NPHIs, in line with the functions and authorities defined and delegated by their parent organization.

Table 2. Summary of four arrangements for public health governance

NPHI as parastatal agency	NPHI as line agency of the ministry of health	Public health within the ministry of health, without an NPHI	Interministerial coordinating body
Description			
A separate institute (arm-length body) from the ministry of health responsible for public health governance defined by legislation or equivalent. This body could report to a board of governors or council. Some countries may have more than one entity responsible for various EPHFs and services.	An institute under the ministry of health and reporting directly to the ministry of health responsible for public health governance as defined by the parent body and associated legislation. Some countries may have more than one entity directly reporting to parent organizations.	The ministry of health is responsible for delivery of health services as well as leading delivery and coordination of public health functions. There is no clearly identifiable separate public health institute or equivalent.	Authority of an interministerial coordinating body is amplified to govern public health, particularly in times of crisis. Ad hoc arrangement activated for multi-agency input to streamlined and time-bound response.
Financing			
Usually have more financial and administrative flexibility. Public health budget decided from the ministry of health with degree of flexibility in utilization and additional resource mobilization.	Usually the ministry of health budget, with dedicated funds for public health as defined by the parent body.	The ministry of health budget. Budget lines for public health subject to the ministry of health decision.	Centralized funding focused on specific emergency context while resources from allied line ministries are also brought together.
Examples of countries			
Liberia, Canada	Nigeria, United States of America	Qatar, Thailand	United Kingdom COVID-19 response

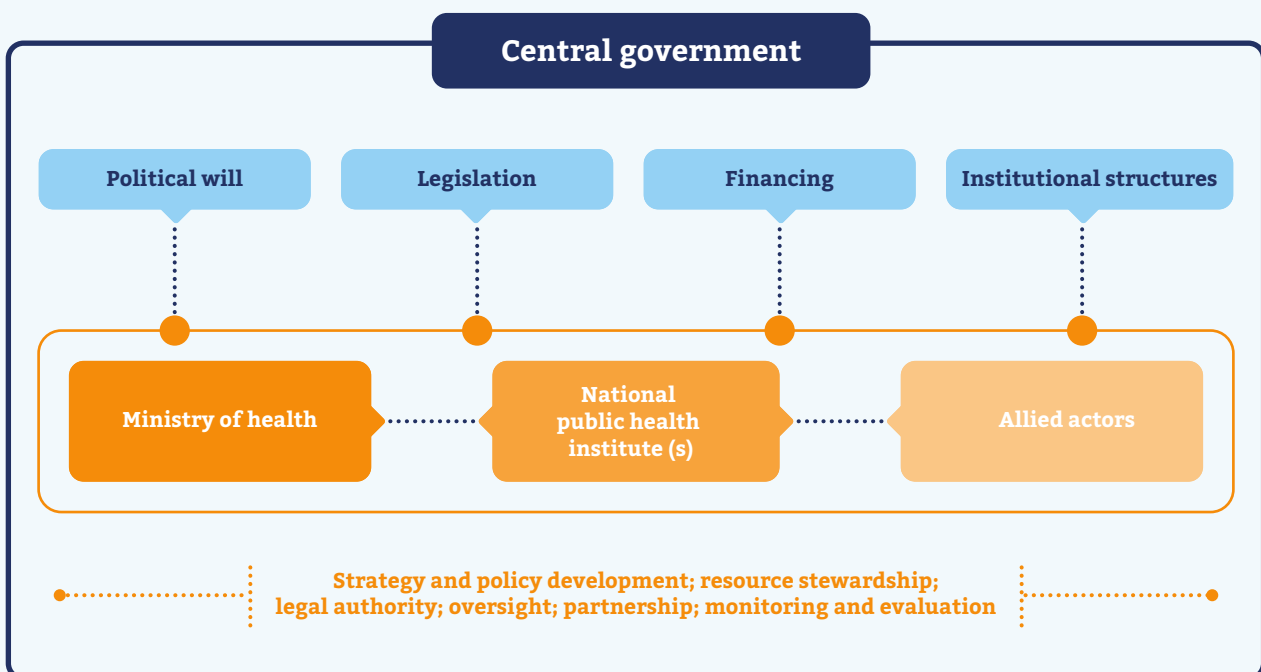
Arrangement 1: National public health institute or institutes as parastatal or arm-length body

An autonomous national public health institute (or institutes) (Figure 4) could be the subject of a clear government statute legitimizing it as an institution to lead public health matters and have a budget line within the government's national budget. This is the case with the National Public Health Institute of Liberia, where the mandate for the institute is built into the National Public Health Institute of Liberia Act (2016) (33).

An autonomous institute operates with legal authority to make executive decisions on matters related to operationalizing EPHFs, at least for some of the whole scope of public health functions. With its level of authority and autonomy, the entity has oversight of and leads in strategy and policy

development to guide public health processes, manage funding use for public health, coordinate and collaborate with health and allied sectors, and strengthen global health partnerships. "Autonomy" entails some degree of financial and administrative flexibility, which can be considered an advantage in its operations, including implementation of routine public health actions and emergency preparedness, deploying and managing rapid crisis response, enforcing public health policies and building public trust (48). However, the scope of public health governance in this arrangement can still be dependent on legislation that underpins its operation and funding allocation.

Figure 4. Schematics of autonomous public health institute or institutes-led governance as parastatal or arm-length body



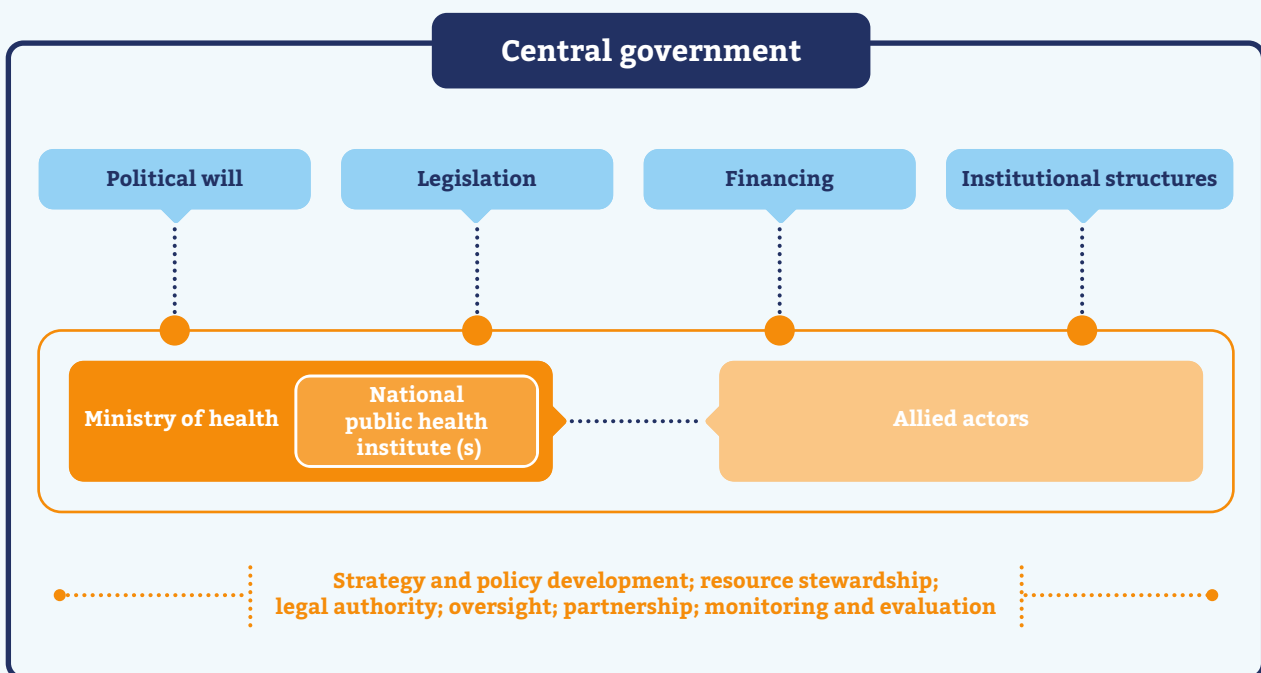
Arrangement 2: Semi-autonomous national public health institute or institutes as line agency (executive agency or body)

In this arrangement, public health is governed by one or more designated national public health institutes embedded within the ministry of health, while the ministry of health possesses oversight and legal authority over them (Figure 5). For example, the Nigeria Centre for Disease Control is responsible for public health governance for designated areas of EPHFs and is under the direct authority of the Federal Ministry of Health and Social Welfare (33). However, the Nigeria Centre for Disease Control, through the legal reforms instituted after the COVID-19 pandemic, has “increased autonomy” from the ministry of health, enhancing its authority to prevent, detect and respond to public health threats (49).

A national public health institute under the ministry of health typically ensures centralized oversight, policy alignment, and standardized public health implementation. The ministry of health retains legal authority, funding control and direct

political accountability, while integrating the public health institute’s workforce into the national health system. The national public health institute, under the leadership of the ministry of health, is responsible for defined intersectoral coordination for public health. This structure enables coordinated decision-making, resource allocation and capacity-building within a defined health framework. This arrangement supports policy coordination, rapid emergency response and shared resource utilization. It can also enhance data-driven decision-making, strengthen health system integration and facilitate international collaboration, which are roles seen in many national public health institutes (50). However, scope of authority to govern public health along with scientific and operational independence under this arrangement is defined by the parent organization and is subject to the size of funding allocation.

Figure 5. Schematics of semi-autonomous public health institute or institutes



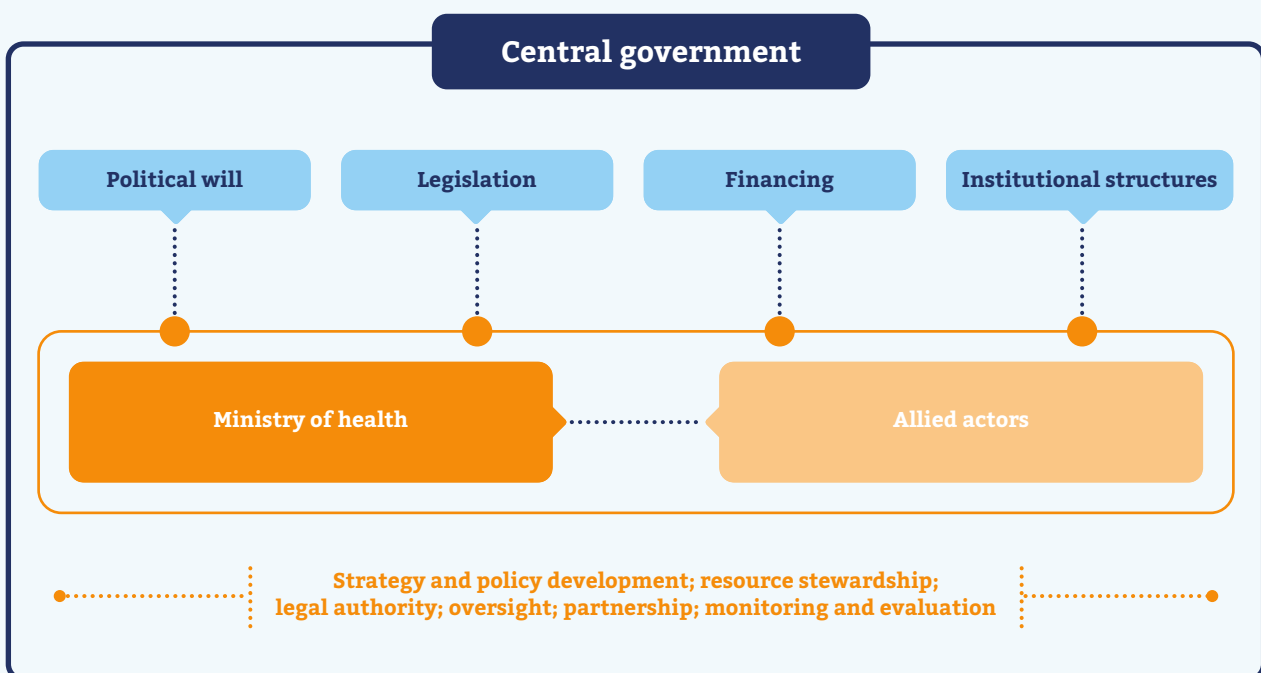
Arrangement 3: Public health within the ministry of health

In this arrangement, the ministry of health leads public health governance and there is no designated separate or independent national public health institute (or institutes) (Figure 6). A centralized public health system under the ministry of health integrates public health, health care services and policy formulation within a single structure (51). For example, Qatar's public health governance is led by the Ministry of Public Health, which is responsible for overseeing and regulating the country's public health and the broader health care system (52).

The ministry of health has direct political oversight, manages public health funding, and leads routine and emergency response efforts, ensuring alignment with national priorities. This arrangement can enable streamlined coordination of health care services, integration of public health infrastructure, and policy-driven decision-making based on government priorities and development goals.

This arrangement strives for uniform health policies, reducing fragmentation between public health and health care services within the scope of the ministry of health. It can enable the ministry of health to achieve easier integration of the public health programme within health care delivery, and resource sharing of interrelated priorities according to its preference. Additionally, this arrangement enables streamlined resource allocation and rapid political decision-making in emergencies, ensuring swift mobilization of resources for public health interventions. The arrangement also entails scientific and operational independence for public health as matters of authority of the ministry of health. However, under this arrangement, the visibility of public health and accountability lie under the auspices of the ministry of health.

Figure 6. Schematics of public health within the ministry of health and no dedicated national public health institute or institutes



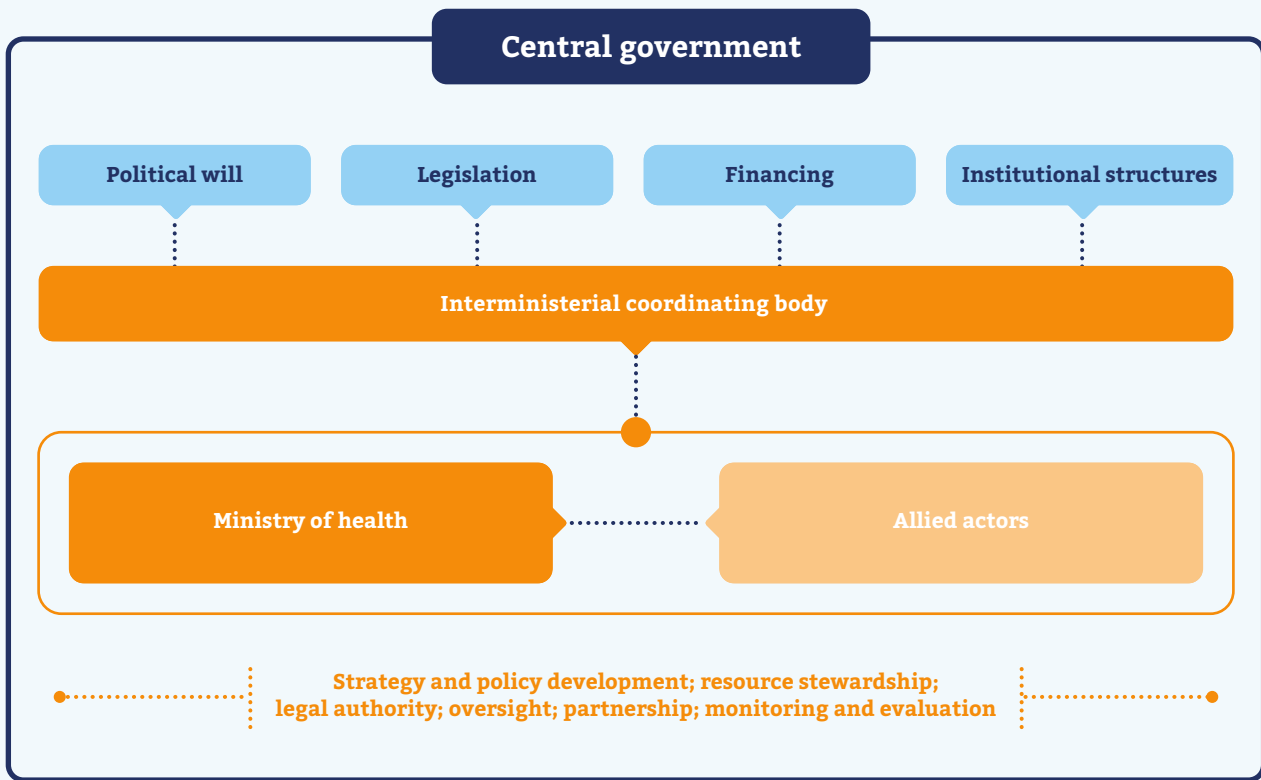
Arrangement 4: Interministerial coordinating body

Countries using arrangements 1–3 above may employ an ad hoc intersectoral coordinating body (for example, a platform, an office) whose legal authority can be magnified or scaled up during escalating public health emergencies. This interministerial body led by the central government office serves as the lead coordinating entity in crisis settings, facilitating policy alignment among health and non-health sectors to improve routine and emergency public health responses (Figure 7). Aspects of public health under authorities outside the ministry of health or public health institute are monitored through the interministerial entity, in which the ministry of health or NPHI plays a substantive role as the principal authority for public health. Decision-making is collective and is made through joint oversight comprising representatives from both health and non-health sectors spearheaded by the central government.

Apart from the crisis context, this type of interministerial or departmental coordinating body is present in countries such as Australia, where the highest level of government spearheads initiatives to strengthen the national commitment to public health and enhance coordination across various sectors. The arrangement promotes cross-sectoral collaboration, integrating policies from non-health sectors to address the broader

determinants of health (53). Through integrated policy-making, health considerations are systematically embedded in decision-making across all sectors to maximize positive outcomes and prevent adverse health impacts (12). Additionally, this arrangement fosters shared accountability, ensuring that all sectors take responsibility for public health, leading to a unified and coordinated approach to reducing health disparities and improving overall well-being. For example, in 2007, South Australia implemented Health in All Policies by positioning it as a core government process, focusing on central governance and accountability, and developing a “health lens” analysis process (38). The Department of Health and Ageing (now known as SA Health) was the primary entity responsible for public health within the Health in All Policies initiative. This department was responsible for providing health expertise, leadership and guidance; facilitating cross-sectoral collaboration through health lens analysis; ensuring integration of public health considerations into broader governmental policy-making; and coordinating evaluation and ongoing improvement of Health in All Policies practices. SA Health’s leadership was central to maintaining health as a priority across governmental sectors (38).

Figure 7. Schematics of central government-led governance with interministerial coordinating body



2.5 Application of the public health governance framework (including guiding principles, core components and enablers)

There is no one governance framework that could fit well in all countries. The scope of this study is not to offer a comparative evaluation of different institutional frameworks or suggest any single arrangement as the preferred option (Figure 8).

The lead institutions, however, are required to ensure that the identified core components and principles are embedded in their establishment and day-to-day public health operations, and critically tested and improved before and when a crisis occurs. In each of the institutional governance arrangements, the roles of the central government, ministry of health and NPHIs are key to driving the public health agenda across all relevant government functions (including beyond the health sector) and ensuring accountability

mechanisms for multisectoral inputs in delivery of EPHFs in line with population health needs (Box 2).

Application of the enablers and core components must be guided by the three principles (integration, comprehensiveness and shared responsibility) to fulfil public health governance roles from an EPHF perspective. The three principles can be applied and demonstrated by each of the institutional arrangements described in this section. However, there is often no uniformity, clarity or predictability in how they are applied within or across the various types of governance arrangements at national level. It appears that the extent to which any of the governance arrangements leads, supports and provides

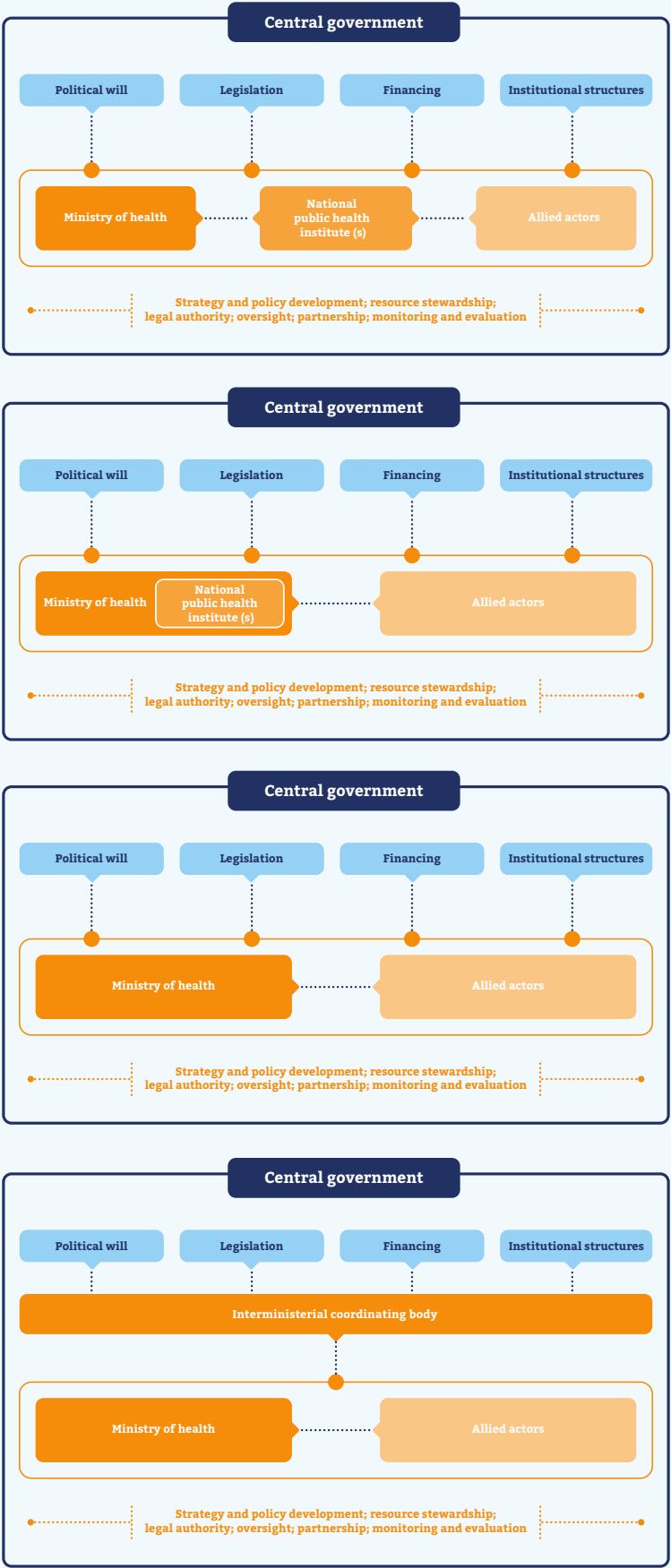
stewardship for the delivery of EPHFs depends on the incorporation of these principles in their mandates, structuring and resourcing. For example, where the mandate of an entity only covers response to public health emergencies without explicitly requiring interlinkages with other aspects of public health, it is unlikely to have the authority, capacity or incentive to foster integration or alignment with other priorities, such as health promotion, disease prevention and health protection. Also, by applying the principle of shared responsibility, the roles and responsibility of the various governance structures involved in delivery of the EPHFs would be more clearly defined, avoiding duplication, gaps and lack of clarity with regard to who is accountable for what in public health within health and allied sectors.

Given the different stages of maturity of public health systems and socioeconomic development in countries, it is critical that each of the current governance arrangements undertake a review of its operation with reference to the identified core components, enablers and guiding principles by using EPHFs as a framing for public health. This review should be a routine requirement (for example, every three to five years) and can also be done after multi-agency exercises or in response to disruptive public health events. The current and future burden of diseases, risk profile and population health needs assessment should also inform the identification of gaps and requirements for improvement, irrespective of the institutional arrangement for public health governance in the country or specific setting (5).

The results of public health reviews should inform central authorities (for example, heads of government, cabinet office facilitating interministerial coordination) in establishing and institutionalizing shared actions and accountability by stakeholders at all levels, including national and subnational levels. For example, shortcomings in the legislation, accountability mechanisms for multisectoral inputs, financing for public health, and performance monitoring need to be effectively addressed to ensure functional public health governance facilitated by an enabling environment from the central government. Reviews and reforms with a focus on public health governance should also entail close collaboration between all stakeholders that influence and contribute to EPHFs, explicitly defining roles and responsibilities, and ensuring that public health is afforded its due attention and integrated delivery. In addition, it would be crucial to review the proportionality of investments in public health (compared to clinical care), including the availability of dedicated national budget allocations to ensure that public health is a priority backed by adequate resources during routine and crisis periods. Robust monitoring and evaluation tools and resources would also be critical to demonstrate the health impacts of (indirect or long-term) policies across sectors and maintain political commitment.

»»» There is no one governance framework that could fit well in all countries. The scope of this study is not to offer a comparative evaluation of different institutional frameworks or suggest any single arrangement as the preferred option.

Figure 8. Schematics of four existing institutional set-ups in reference to public health



Box 2. Examples of generic functions of central government, ministry of health and NPHIs to facilitate public health governance

Central government

(health and non-health ministries under the leadership and oversight of the head of government or cabinet office):

- drives cross-sectoral collaboration to deliver shared sectoral goals promoting population health;
- embeds public health as a central priority within overall government strategy;
- ensures oversight and accountability for effective intersectoral working.

Ministry of health:

- provides oversight and policy responsibility for the health sector;
- plays an enhanced role in stewarding strategic public health action in partnership with other sectors and government departments.

National public health institution:

- well placed to provide a coordinating role for EPHFs but requires an expanded remit (for example, wider focus to include health promotion and disease prevention), robust legal foundations, and flexible and sustainable reporting and resourcing, while preserving scientific independence.

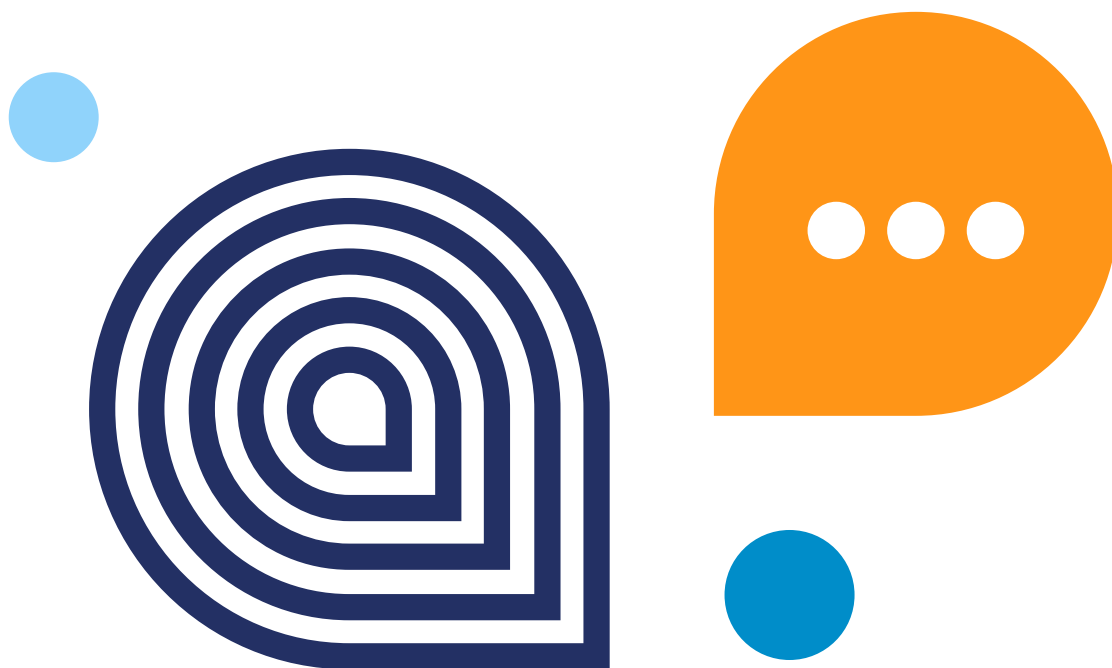
Global health actors can support strengthening of country-level governance by developing and updating international legal instruments that ensure clearer roles, responsibilities and mechanisms of cooperation, providing guidance and technical support, and through funding with the aim of developing governance capacity at country level (for example, investment in institutions).



Given the different stages of maturity of public health systems and socioeconomic development in countries, it is critical that each of the current governance arrangements undertake a review of its operation with reference to the identified core components, enablers and guiding principles by using EPHFs as a framing for public health.



3. Discussion



There is global acknowledgement of the importance of public health governance for delivering EPHFs and public health services. However, the approaches and structures applied vary considerably between and within countries. The lack of consistency in approach and structure for public health governance can be attributed to several factors, including the variability between countries with respect to legal foundations, intersectoral governance structures, health care institutions, and the degree and form of centralization of government structures (for example, federated or unitary systems) ([10](#), [23](#), [54](#)). Public health governance is also influenced by the evolving contexts and global trends to which governments and societies must respond, including changes in donor and government priorities, fiscal constraints, geopolitical factors, changing demographics and public health crises. These complexities and challenges further underscore the need to rethink current approaches to public health governance, considering its importance for the delivery of all other public health functions and services.

The definition of public health governance is not evident in the literature but is important. The purpose of a dedicated definition of public health governance, and its framework, is to address prevailing fragmentation and siloed approaches to public health and promote public health as a matter of shared intersectoral accountability at all administrative levels. Its overlap and complementarity with health system governance (usually under the ministry of health) calls for better integration, with clear mapping and definition of the roles of the ministry of health, NPHIs and allied sectors to ensure that public health functions and services are governed and delivered comprehensively. A clear and comprehensive definition of public health governance is essential to foster effective multisectoral responsibility and coordination, recognizing that the implementation of EPHFs extends beyond the health sector.

In the face of evolving threats, effective governance for public health in both routine contexts and during crises has never been more critical and must be intentionally established and institutionalized within and across health and allied sectors based on the EPHFs. Governance for public health needs to be established in a period of routine operation. This can be achieved through coordinating the implementation of the EPHFs and public health services, for example through the application of health promotion instruments of engagement, empowerment, health literacy, and Health in All Policies in governance processes. Those actions will help to build the processes, trust and relationships needed to manage acute and complex public health events or emergencies. During crises, it is important that attention is also given to other service-oriented EPHFs (such as health promotion, disease prevention, health protection, and public health monitoring and surveillance) aside from the emergency at hand. As evidence suggests, public health governance is inherently multisectoral, and rethinking governance for public health will require a structured approach that takes into consideration the identified core components, enablers and guiding principles, engaging all actors and sectors to deliver EPHFs in a comprehensive and coordinated manner. Highly complex coordination structures forged during a crisis with heads of government are not sustainable for the governance of EPHFs during peace times and are also not an optimal structure to repeat in future crisis contexts. Moreover, a siloed approach to governance for certain EPHFs (such as emergency response and preparedness) can compromise scope for sector wide collaboration, capacity development and interdependence between and within service-oriented and enabling EPHFs.

Public health governance cannot be confined to the health sector alone. The interconnected nature of public health necessitates whole-of-government and whole-of-society approaches that integrate policies across sectors, ensuring that public health considerations are embedded in all facets of governance. Legislation that defines the scope of public health, including the roles of various actors during routine and crisis periods, is essential. Countries should establish clear authority and shared accountability mechanisms to navigate routine public health operations and scale responses effectively during crises. Even in the context of public health reforms, with consolidation of functions and services, it is impractical to expect that all EPHFs would fall solely under one entity, as public health is a shared responsibility that can be incorporated in the policies of all sectors (Health in All Policies). This necessitates establishment of functional coordination as part of public health governance during times of “normalcy”, namely, routine public functions to establish trusted working relationships and mutual understanding and accountability across sectors on public health matters, manage public health issues early, synergize efforts, and make coordination of service delivery and emergency response (when that occurs) more streamlined and efficient.

Public health governance cannot be confined to the health sector alone.

While there is no singular governance arrangement that fits all contexts, strengthening the core components of governance through clear legal mandates, sustainable financing, institutional frameworks, and political will is essential for achievement of resilient public health systems (comprising the core health and allied sectors outside the ministry of health) that are equipped for the comprehensive delivery of EPHFs. It can be argued that many countries have already defined many of the key components and principles of public health governance, if not all; but effective public health governance requires the comprehensive incorporation of these underpinning principles (integration, comprehensiveness and shared accountability) in all elements of the organization and delivery of the EPHFs. The gaps in application of the key principles can be attributed to a number of

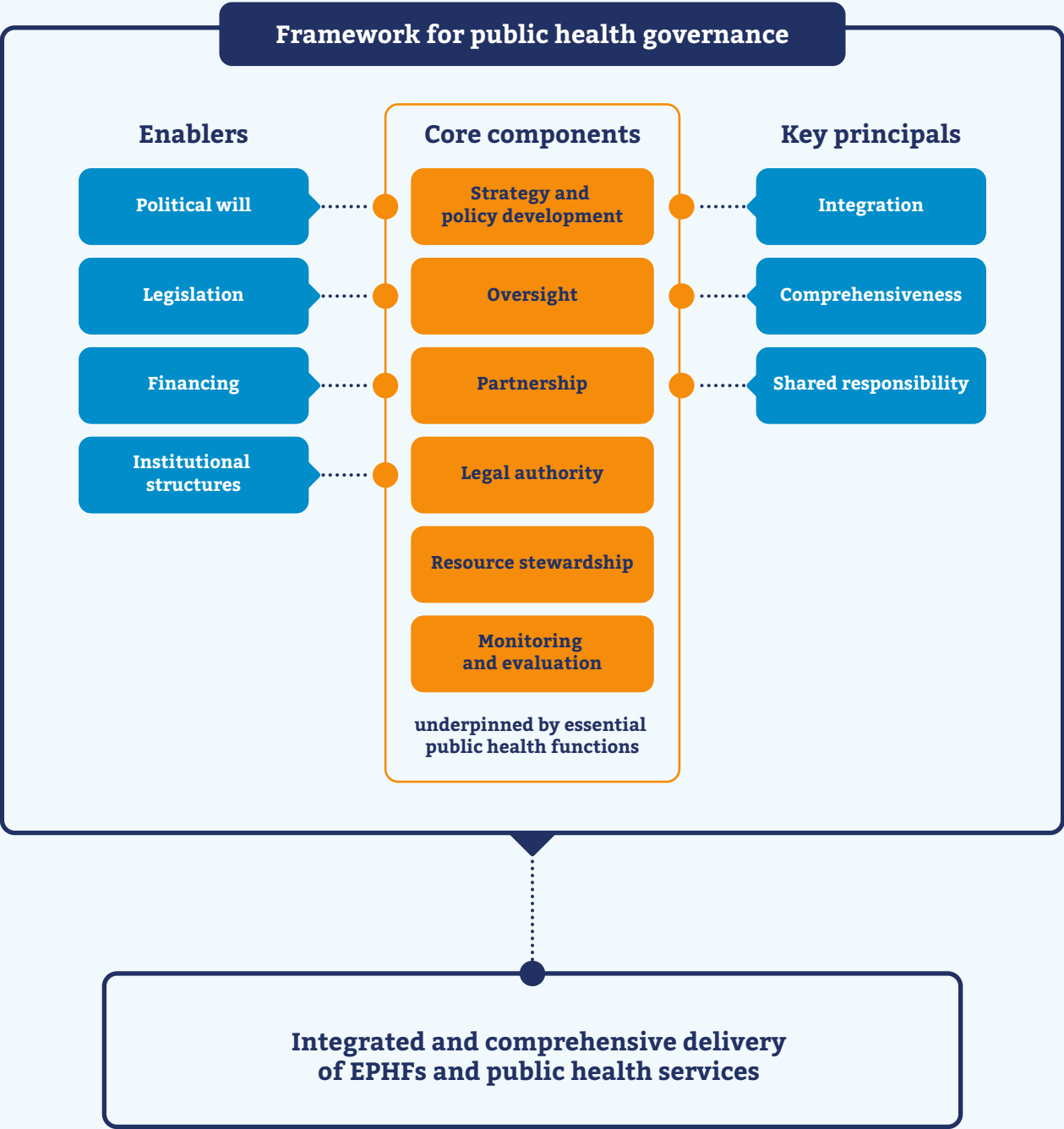
factors, including lack of political will, siloed and fragmented funding and accountability mechanisms, and inadequate understanding of public health through the lens of the EPHFs. Political commitment, informed by evidence and conceptual clarity about the EPHFs, is needed to establish and sustain governance structures and linkages that are mandated and empowered to ensure the integrated and comprehensive delivery of EPHFs with shared responsibility of stakeholders within and beyond health sectors.

To support application, a checklist (Table 3) and a framework (Figure 9) are presented as practical tools to guide the approach to benchmarking and further improve public health governance by applying the recommended enablers, components and principles.

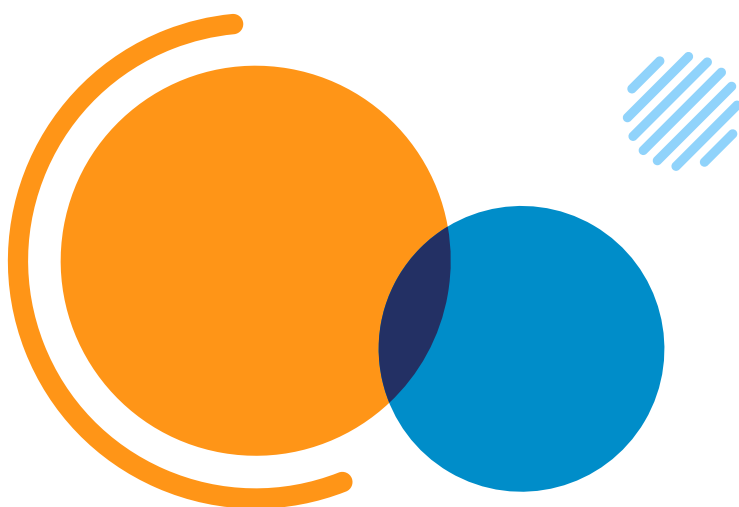
Table 3. High-level checklist for countries to benchmark the current status and build more robust public health governance

Checklist	
☐	Public health act (with statutory guidance) or equivalent is in place with: • clear articulation of scope of public health using WHO and IANPHI list of EPHFs; • defined roles and responsibilities related to the delivery of EPHFs for health and allied sectors; • requirement for reviews of the public health act in reference to institutional set-up in the delivery of EPHFs in line with current and future burden of diseases and population health needs.
☐	There is a designated entity or mechanism for public health agreed by the government responsible for delivery, coordination and oversight of EPHFs.
☐	Sufficient and sustainable budget is allocated to public health (whether through individual national budget line item, ministry of health budget, or shared budget with other sectors).
☐	There is a designated public health focal point in health service delivery and all allied sectors, to support coordination with dedicated entity for public health.
☐	Public health is identified as a priority by head of government along with stakeholders in allied sectors, to support a Health-in-All-Policies approach.
☐	Comprehensive approach to monitoring and well instituted accountability mechanisms for public health governance are established in reference to operation, scientific independence, coordination, and public health service delivery.

Figure 9. Framework for effective public health governance



4. Conclusion



There are significant variations in governance arrangements for public health between countries. Success in public health governance is contingent on well defined roles and responsibilities, streamlined coordination, and an adaptive approach that fosters collaboration between health and allied sectors, enabling the core components to be achieved. To move forward, it is critical that governance for public health is designed to ensure the delivery of all EPHFs in line with population health needs in routine periods as well as during emergencies. There is an urgent need for countries to review and change the status quo where necessary to reposition public health as a priority with routine (not just crisis-driven) intersectoral accountability.

Governments should prioritize investment in public health governance as a whole-of-government, whole-of-society approach, recognizing it as a foundational pillar of national security and socioeconomic stability. Strong governance not only enhances capacities for public health emergency management but also fosters public trust and ensures equitable health outcomes in all contexts. Robust governance mechanisms are also essential to foster evidence-informed public health planning and policymaking processes. As countries and the global community navigate an increasingly complex and fiscally constrained health landscape, a commitment to reinforcing and strengthening governance structures will be instrumental in safeguarding public health as a premium for a stable and prosperous future for generations to come.

Success in public health governance is contingent on well defined roles and responsibilities, streamlined coordination, and an adaptive approach that fosters collaboration between health and allied sectors, enabling the core components to be achieved.

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Annex 1. Methodology

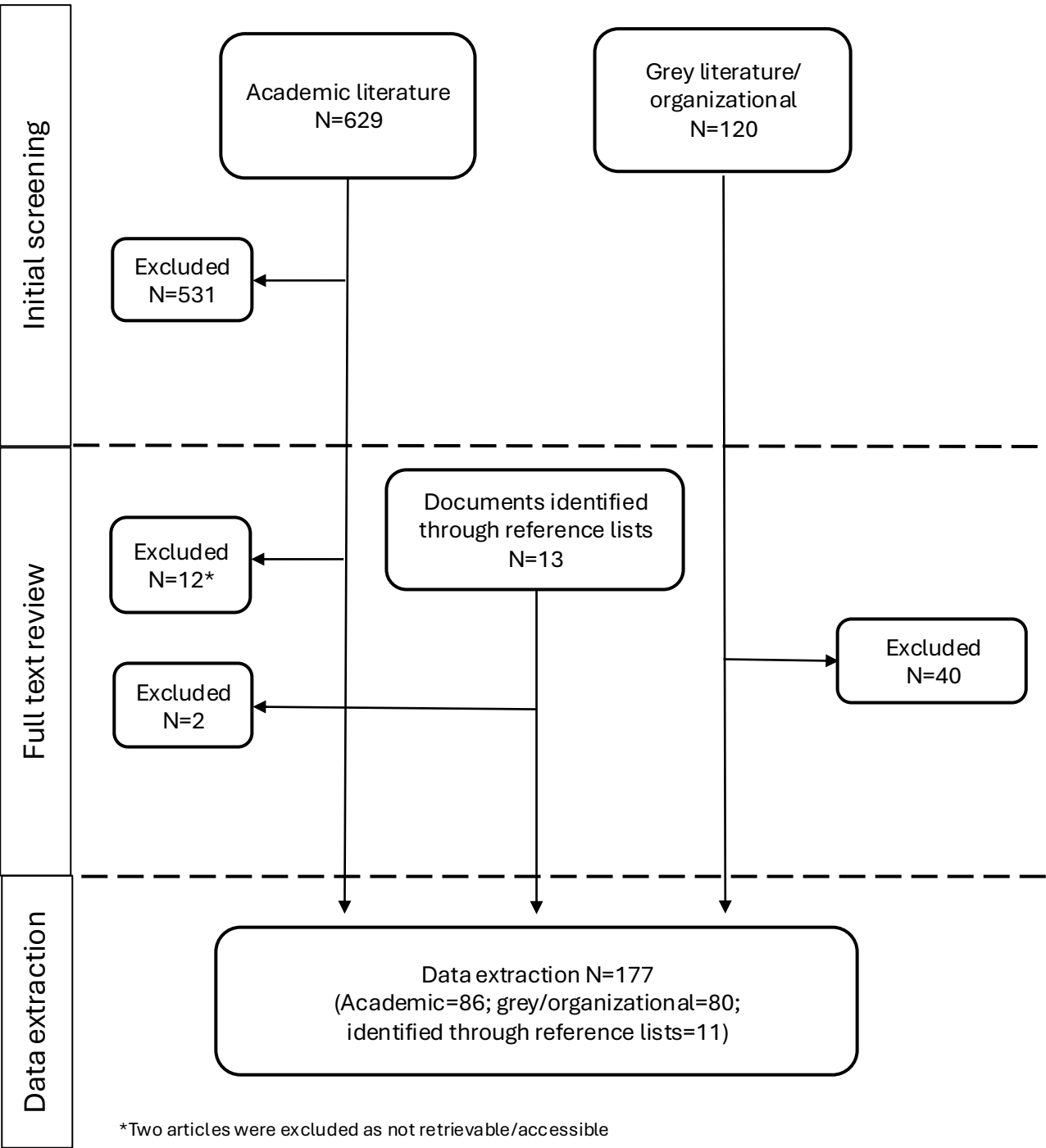
Search terms

PubMed search terms	Embase search terms
("Public Health"[Title] OR "Population Health"[Title] OR "Essential public health"[Title]) AND (public health administration[MeSH Terms] OR "function*" [tiab] OR "system" [tiab] OR "systems" [tiab] OR "structure" [tiab] OR "structures" [tiab] OR "organization*" [tiab] OR "delivery" [tiab] OR "operat*" [tiab] OR "authority" [tiab] OR "authorities" [tiab] OR "Institution*" [tiab] OR "Agency" [tiab] OR "Agencies" [tiab] OR "Executive" [tiab] OR "Executives" [tiab] OR "Model" [tiab] OR "Models" [tiab]) AND ("Governance" [tiab] OR "admin*" [ti] OR "Stewardship" [ti] OR "Oversight" [ti]) Filters: English, from 2000–2023	('public health':ti OR 'population health':ti OR 'essential public health':ti) AND ('public health service'/exp OR 'public health service' OR 'function*':ti,ab,kw OR 'system':ti,ab,kw OR 'systems':ti,ab,kw OR 'structure':ti,ab,kw OR 'structures':ti,ab,kw OR 'organization*':ti,ab,kw OR 'delivery':ti,ab,kw OR 'operat*':ti,ab,kw OR 'authority':ti,ab,kw OR 'authorities':ti,ab,kw OR 'institution*':ti,ab,kw OR 'agency':ti,ab,kw OR 'agencies':ti,ab,kw OR 'executive':ti,ab,kw OR 'executives':ti,ab,kw OR 'model':ti,ab,kw OR 'models':ti,ab,kw) AND ('governance':ti,ab,kw OR 'admin*':ti OR 'stewardship':ti OR 'oversight':ti) AND [2000–2023]/py AND [english]/lim

Inclusion and exclusion criteria

Inclusion criteria	Exclusion criteria
Literature on national/ subnational public health governance and delivery models.	Literature considering disease-specific approaches.
	Literature relating primarily to global public health governance (i.e. not national or subnational level public health governance).
	Literature relating to health service delivery governance specifically but not EPHF provision (e.g. articles on hospital service governance).
	Any other literature not containing data specific to the literature review's aim/question.

PRISMA results



Annex 2. Summary of definitions of terms relevant to public health governance, from literature

Term	Description	Reference(s)
Collaborative governance	A governing arrangement where one or more public agencies directly engage non-state stakeholders in a collective decision-making process that is formal, consensus oriented and deliberative and that aims to make or implement public policy or manage public programs or assets.	(1, 2)
Collaborative governance	The processes and structures of public policy decision making and management that engage people across the boundaries of public agencies, levels of government and/or the public, private and civic spheres to carry out a public purpose that could not otherwise be accomplished	(1, 3)
Governance	A collective process involving a group of stakeholders and it embraces the setting of strategic direction and objectives; making policies, laws, rules, regulations, or decisions; raising and deploying resources to accomplish the strategic goals and objectives; and ensuring that strategic goals and objectives are accomplished.	(4)
Governance	A new process of governing; or a changed condition of ordered rule; or the new method by which society is governed.	(5, 6)
Governance	A set of processes (customs, policies or laws) that are formally or informally applied to distribute responsibility or accountability among actors of a given [health] system.	(7)
Governance	Actions and means to promote collective action and deliver collective solutions, or as self-organizing, interorganizational networks that are interdependent.	(8)
Governance	Ensuring that strategic policy frameworks exist and are combined with effective oversight, coalition building, regulation, attention to system-design and accountability.	(9, 10)
Governance	How governments and other social organizations interact, how they relate to citizens, and how decisions are taken in a complex world. Thus governance is a process whereby societies or organizations make their important decisions, determine whom they involve in the process and how they render account.	(11-13)
Governance	How societies make and implement decisions. It is the ensemble of rules, formal and informal, that decision-makers and implementers follow when acting.	(14, 15)
Governance	The ability of government to develop an efficient, effective, and accountable public management process that is open to participation and that strengthens rather than weakens a democratic system of government.	(16)

Term	Description	Reference(s)
Governance	The exercise of political, economic and administrative authority in the management of a country's affairs at all levels, comprising the complex mechanisms, processes, relationships and institutions through which citizens and groups articulate their interests, exercise their rights and obligations and mediate their differences.	(17)
Governance	The exercise of political, economic and administrative authority in the management of an organization's affairs at all levels.	(18)
Governance	The general procedures that governments within a country use to make and implement decisions.	(19)
Governance	The manner in which power is exercised in the management of a country's economic and social resources.	(20)
Governance	The process and institutions through which decisions are made and authority in a country is exercised	(15, 21)
Governance	The process of continuing interaction between participants inside and outside the formal structures of government, bringing to bear a diversity of frameworks through which matters are seen to need attention and responses evaluated.	(8, 22)
Governance	The process of creating an organizational vision and mission —what it will be and what it will do - in addition to defining the goals and objectives that should be met to achieve the vision and mission; of articulating the organization, its owners and the policies that derive from these values - policies concerning the options that its members should have in order to achieve the desired outcomes; and adopting the management necessary for achieving those results and a performance evaluation of the managers and the organization as a whole.	(23)
Governance	The process through which governments and other social organizations interact, relate to citizens and take decisions in an increasingly complex and interdependent world.	(24)
Governance	The rules, processes, and behavior by which interests are articulated, resources are managed, and power is exercised in society.	(25)
Governance	The structures that bring actors together and the actions flowing from their mutual engagement and deliberations (i.e., the agreement to frame policies in a particular manner, the decision to adopt particular policies, use particular policy instruments to effect implementation, etc.)	(8)
Governance	The system of decision-making whereby directions are set, authority is exercised, and events are monitored and managed. Governments that recognize the complexity of social and economic factors will govern through engagement with market and civil society actors in policy development and implementation. Governance may include action that goes well beyond government by delegating policy formulation and policy implementation or parts of it to stakeholders or stakeholder organizations. In essence governance is about power relationships.	(8)
Governance	The traditions and institutions by which authority in a country is exercised for the common good, including the processes by which those in authority are selected, monitored and replaced; the capacity of the government to effectively manage its resources and implement sound policies; and the respect of citizens and the state for the institutions that govern economic and social interactions among them.	(26, 27)

Term	Description	Reference(s)
Governance	Ways of managing the course of events in a complex social system.	(28)
Governance for health	Governance for health promotes the joint action of health and non-health sectors, of public and private actors and of citizens for a common interest. It requires a synergistic set of policies, many of which reside in sectors other than health as well as outside of government and need to be supported by structures and mechanisms that enable collaboration. It gives strong legitimacy to health ministers and ministries and public health agencies to reach out and to perform new roles in shaping policies that promote health and well-being.	(29)
Governance for health	The attempts of governments and others to steer communities, whole countries or groups of countries in the pursuit of health and well-being as a collective goal. Governance for health promotes joint action of health and non-health sectors, of public and private actors and of citizens for a common interest. It requires a synergistic set of policies, many of which reside in sectors other than health as well as sectors outside government, which must be supported by structures and mechanisms that enable collaboration. It gives strong legitimacy to health ministers and ministries and to public health agencies, to help them reach out and perform new roles in shaping policies to promote health and well-being.	(11, 30)
Governance for health	The joint action of health and non-health sectors, the public, private sector and citizens in common interest.	(31)
Governance for health	Those rules for solidarity to achieve engagement of all the actors (governments and other actors) in the response to the epidemic through both whole-of-government and whole-of-society approaches.	(11, 32)
Governance for health	The attempts of governments and other actors to steer communities, whole countries or even groups of countries in the pursuit of health as integral to well-being.	(31)
Governance for health and well-being	The steering of “communities, whole countries or even groups of countries in the pursuit of health as integral to well-being through both whole-of-government and whole-of-society approaches”.	(33-35)
Health governance	Actions and means a society adopts to organize itself for promoting and protecting the health of its population.	(11, 36)
Health governance	The governance of the health system and health systems strengthening.	(31)
Health system governance	Governance undertaken with the objective to protect and promote the health of the people. Governance involves (1) setting strategic direction and objectives; (2) making policies, laws, rules, regulations, or decisions, and raising and deploying resources to accomplish the strategic goals and objectives; and (3) overseeing and making sure that the strategic goals and objectives are accomplished.	(37, 38)
Health systems governance	Actions and means adopted by a society to organize itself in the promotion and protection of the health of its population.	(36)
Health systems governance	The processes, structures and institutions that are in place to oversee and manage a country's health system.	(39)

Term	Description	Reference(s)
Intersectoral governance actions	Actions facilitated by intersectoral governance structures that aim to align other governance policies with health objectives. Examples of different intersectoral governance actions include evidence support, setting objectives, goals and targets, coordination, advocacy, monitoring and evaluation, policy guidance, financial support, providing legal mandates, implementation and management. They therefore range from rather “soft” to “hard” interventions and cover all stages of the policy cycle.	(8)
Intersectoral governance structures	Structures that exist to facilitate the collaboration between different ministries, departments or sectors. Intersectoral structures are “tangible” or “visible” in terms of leaving a trace in the organigram or prescribing distinct entities or procedures inside government and administration. Intersectoral governance structures are in this respect different from collaboration based merely on personal relations. Intersectoral structures can be owned or co-owned by the ministry responsible for health or by the whole government. Also included are other ministries’ intersectoral governance structures to the extent they are accessible to the ministry of health.	(8)
Intersectoral governance	One or more sectors working together for a common goal.	(40)
Intrasectoral governance	Governance for health and well-being within a single sector.	(40)
Leadership and governance	Ensuring that a strategic policy framework exists and is combined with effective oversight, coalition building, regulation, attention to system-design and accountability.	(38, 41)
Multisectoral governance	Multiple sectors working independently for a common goal.	(40)
System of governance	An enabling system with improved health and well-being integrated as an expected outcome of the system.	(40)

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